

HEALIO / QUIIKCARE – OFFSHORE TEAM MVP BRIEF

Project Type: Web Application (MVP)

Delivery Deadline: Maximum 10 weeks

Preferred Delivery: 6–8 weeks

Build Focus: Functionality, compliance, and auditability over polish

1. Project Objective

Build a lean, functional MVP for a care staffing marketplace.

The goal is to validate core workflows while meeting real-world safeguarding expectations.

2. User Roles

Care Provider (Employer)

Care Worker (Carer)

Admin (Platform Owner)

3. Core MVP Features (Must Be Delivered)

Authentication: Email signup/login, password reset, role selection.

Profiles: Worker and Provider profiles with ratings and compliance status.

Shift Management: Providers post shifts; workers accept shifts; lock on acceptance.

Payments: Integrate Stripe Connect for marketplace payments. Platform fee applied.

Stripe Costs: All Stripe subscription, transaction, and usage costs are borne by the Client.

Developer scope is limited to technical integration only.

Payouts: Manual admin-controlled payout status (Pending / Paid).

Messaging: Text-only, shift-linked messages (max 500 characters).

Ratings & Reviews: Post-shift 1–5 star rating with optional text review.

Admin Dashboard: Users, shifts, payments, compliance status, moderation.

4. DBS Compliance – MVP Approach (Core Requirement)

Carers must declare DBS status during signup and confirm possession via a mandatory checkbox.

DBS status shown as: Declared / Physically Verified / Rejected / Expired.

For the first shift only, the carer must attend with a physical DBS certificate.

The care provider must visually verify the DBS certificate at the first shift.

Care provider confirms verification via a checkbox in the platform.

Provider confirmation updates DBS status to Physically Verified.

The system must store timestamp and provider identity for audit purposes.

No DBS documents or certificate images are to be stored in the MVP.

5. False DBS Declaration Handling (MVP)

If a carer is found to have falsely declared possession of a valid DBS certificate, Admin may mark the status as False Declaration.

The system must allow Admin to record a surcharge equal to 50% of the cost of the affected shift.

Surcharge enforcement is manual at MVP stage and recorded for audit purposes.

Carer acknowledgement of this rule must be captured during signup.

6. Stripe Connect Acceptance Criteria

A test provider payment can be completed successfully.

Platform fee is deducted correctly.

A connected worker account can be created via Stripe Connect.

Admin can manually mark payout status as Paid.

7. Technical Expectations

Responsive web application (desktop + mobile browser).

Clean, readable, well-documented code.

Simple architecture – avoid over-engineering.

Core pages load within ~3 seconds.

Supports ~500 users (MVP scale).

8. Security & Data Handling

HTTPS required.

Passwords must be hashed.

Basic SQL injection and XSS protection.

No storage of raw identity or DBS documents.

All key actions must be timestamped.

9. Infrastructure & Ownership (Non-Negotiable)

Code repository must be under Client ownership.

Hosting must be under Client-owned account.

Developer has access rights only.

Code and data must be exportable.

No proprietary lock-in without approval.

10. Explicitly Out of Scope

Mobile apps.

AI or automated matching.

Multiple DBS or verification providers.

Subscription billing.

Advanced analytics or dashboards.

Push notifications.

NHS Login.

Enterprise features or SLAs.

11. Delivery & Acceptance

Final delivery must be completed within 10 weeks.

Preferred delivery is 6–8 weeks where possible.

All listed features must be functional.

No critical blocking bugs.

Full handover of code, documentation, and access.

12. Working Style Expectations

Weekly progress updates.

Early identification of blockers.

No scope changes without written approval.

Milestone-based delivery approach preferred.

13. Summary for Development Team

This MVP prioritises safeguarding, compliance realism, and deterrence of misrepresentation.

DBS verification includes human confirmation by care providers.

False DBS declarations are logged and handled via admin-controlled surcharge recording.

Phase 2 features are not included.